

Beth Israel Deaconess Medical Center

Anticoagulation Management Clinic

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January 15, 2151

Re: Anticoagulation Management Update

To: Primary Care Provider

Patient: cb70e6ae

Date of Birth: October 8, 2086

MRN: CB-70E6AE-M

Dear Colleague,

I am writing to provide you with an update on the above-named patient's anticoagulation management through the BIDMC Anticoagulation Clinic over the past six months. This 64-year-old male remains under our service for the management of oral anticoagulation therapy related to his diagnosis of paroxysmal atrial fibrillation.

Patient Demographics and Indication

Patient Name:	Patient cb70e6ae
Date of Birth:	October 8, 2086 (Age 64)
Sex:	Male
Indication for Anticoagulation:	Paroxysmal atrial fibrillation (unspecified atrial fibrillation)
CHA₂DS₂-VASc Score:	5 (HTN, DM, prior TIA/stroke, age, CHF/LV dysfunction)
HAS-BLED Score:	2 (moderate bleeding risk)

Current Anticoagulation Regimen

The patient is currently maintained on **apixaban (Eliquis) 5 mg by mouth twice daily**. This dose is appropriate given his renal function (eGFR consistently >30 mL/min/1.73m²), body weight (>60 kg), and age. The patient has demonstrated excellent adherence to therapy and understanding of the medication purpose and administration schedule.

Dosing History and Adjustments

The patient's apixaban dosing has remained stable over the review period. The following table summarizes clinic encounters and any dosing modifications:

Date	Apixaban Dose	Indication / Notes	Action Taken
July 2, 2150	5 mg BID	Routine follow-up post-cardioversion	Dose continued, no change
May 9–10, 2150	Held per protocol	Peri-procedural hold for elective cardioversion	Held AM dose day of procedure
May 8, 2150	5 mg BID	Pre-cardioversion TEE and procedure planning	Continued through evening prior
April 30–May 7, 2150	5 mg BID	Hospitalization for acute kidney injury, monitored	Continued with renal function monitoring
February 4–8, 2150	5 mg BID	Routine follow-up; stable INR not applicable	Dose confirmed appropriate

Peri-Procedural Management

The patient underwent elective cardioversion for paroxysmal atrial fibrillation on May 9, 2150. Per standardized peri-procedural anticoagulation protocols for patients on apixaban and in consultation with the electrophysiology service, the following instructions were provided:

- Continue apixaban 5 mg BID through the evening dose on May 8, 2150 (day prior to procedure).
- Hold morning dose on May 9, 2150 (day of procedure).
- Resume apixaban 5 mg in the evening of May 9, 2150, approximately 6 hours post-procedure, assuming hemodynamic stability and no bleeding complications.
- Transesophageal echocardiogram performed May 8, 2150 confirmed absence of left atrial thrombus.

The patient tolerated the peri-procedural anticoagulation plan without complication. There were no thromboembolic or hemorrhagic events noted in the immediate post-procedure period.

Bleeding History and Adverse Events

Since initiating apixaban therapy, the patient has experienced **no clinically significant bleeding events**. He denies any episodes of:

- Gastrointestinal bleeding (melena, hematochezia, hematemesis)
- Genitourinary bleeding (hematuria)
- Intracranial hemorrhage
- Epistaxis requiring intervention
- Hemoptysis
- Unusual or excessive bruising

The patient did experience a brief hospitalization from April 30 to May 7, 2150, for acute kidney injury (likely pre-renal in etiology related to volume depletion). Apixaban was continued with close monitoring of

renal function. His creatinine improved from peak of 2.1 mg/dL to baseline 1.3 mg/dL by discharge. No dose adjustment was required as his eGFR remained above 30 mL/min/1.73m² throughout.

Renal Function and Laboratory Monitoring

Given the patient's complex medical history including Type 2 diabetes mellitus with diabetic polyneuropathy, hypertension, and recent acute kidney injury, renal function has been closely monitored:

Most Recent Creatinine (July 2, 1.4 mg/dL

2150): 52 mL/min/1.73m² (CKD Stage 3a)

Estimated GFR: Every 3 months or sooner if acute illness

Monitoring Frequency: Current renal function supports continuation of apixaban 5 mg BID.

Dose reduction to 2.5 mg BID would only be indicated if two or more of the following criteria were met: age ≥80 years, weight ≤60 kg, or serum creatinine ≥1.5 mg/dL. Currently only one criterion is met (creatinine 1.4 mg/dL, which is borderline).

Medication Interactions and Concomitant Therapy

The patient's current medication regimen has been reviewed for potential drug-drug interactions with apixaban. Notable medications include:

- Aspirin 81 mg daily (antiplatelet therapy; acceptable for coronary artery disease but increases bleeding risk)
- Metoprolol (no significant interaction)
- Pantoprazole (no significant interaction; provides gastroprotection)
- Insulin, levothyroxine, gabapentin, fluoxetine, and other chronic medications reviewed (no contraindications)

The combination of apixaban with low-dose aspirin is acceptable given his atherosclerotic heart disease, prior coronary angioplasty, and history of TIA. The patient is counseled on enhanced bleeding precautions with dual therapy.

Patient Education and Adherence

The patient demonstrates excellent understanding of his anticoagulation therapy. He verbalizes the purpose of apixaban for stroke prevention, recognizes signs and symptoms of bleeding and thromboembolism, and reports consistent adherence with twice-daily dosing. He utilizes a pillbox organizer and smartphone reminders to ensure timely administration. He has been counseled regarding:

- Importance of adherence and consequences of missed doses
- Recognition of bleeding signs (unusual bruising, blood in urine/stool, severe headache)
- Fall precautions given his obstructive sleep apnea and chronic pain
- When to notify the clinic or seek emergency care
- Informing all healthcare providers of anticoagulation status

Fall Risk Assessment

The patient's fall risk has been evaluated given concerns related to diabetic polyneuropathy, obesity, obstructive sleep apnea (currently on CPAP), and chronic pain. He reports no falls in the past six months. Home safety assessment and physical therapy consultation were recommended and completed during his April–May hospitalization. He ambulates independently with stable gait.

Plan and Recommendations

The patient will continue on apixaban 5 mg by mouth twice daily for stroke prevention in the setting of paroxysmal atrial fibrillation. Our recommendations and plan moving forward include:

1. **Continue current apixaban dosing** without modification at this time.
2. **Renal function monitoring:** Repeat basic metabolic panel every 3 months or sooner if acute illness or medication changes.
3. **Next clinic visit:** The patient is scheduled for anticoagulation clinic follow-up on **April 18, 2021** at 10:00 AM.
4. **Prior to any invasive procedures:** Please contact our clinic at (617) 667-4120 for peri-procedural anticoagulation management guidance.
5. **Consider gastroenterology follow-up:** Given his history of GI procedures (colonoscopy with biopsies) and dual antiplatelet/anticoagulation therapy, periodic surveillance may be warranted. This should be coordinated with his primary care and gastroenterology providers.
6. **Annual reassessment:** CHA₂DS₂-VASc and HAS-BLED scores to be recalculated annually or with significant clinical changes.

Conclusion

This patient has been successfully managed on apixaban therapy for paroxysmal atrial fibrillation with no bleeding complications and excellent adherence over the past six months. His complex medical comorbidities including diabetes, chronic kidney disease, coronary artery disease, and prior TIA make anticoagulation both essential and requiring close monitoring. We will continue to follow him regularly in the Anticoagulation Clinic.

Should you have any questions regarding this patient's anticoagulation management, or if there are changes to his clinical status that may impact therapy, please do not hesitate to contact our clinic at (617) 667-4120. We appreciate the opportunity to collaborate in the care of this patient.

Sincerely,

Margaret R. Sullivan, PharmD, CACP

Clinical Pharmacist, Anticoagulation Management

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CC: Patient chart (MRN CB-70E6AE-M)

Document Date: January 15, 2151

Dictated: January 15, 2151 | **Transcribed:** January 15, 2151

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